

Constipation

Patient Information · Why it matters for pelvic health, and how to fix it

WARWIKI

Constipation means stools that are **infrequent, hard or hard to pass**. It is very common. It matters for pelvic health because **straining** can make prolapse, bladder leakage and pelvic pain worse. The good news: most constipation gets better with simple, proven steps.

About This Condition

A healthy bowel movement passes without straining. Signs of constipation include:

- Going fewer than three times a week
- Hard or lumpy stools, or straining
- A feeling that you have **not fully emptied**
- Needing to press near the vagina or anus to pass stool ("splinting")

What Causes It

- **Too little fiber or fluid**, and not enough activity
- **Ignoring the urge** to go
- **Medicines**, such as opioid pain pills, iron, calcium and some others
- Health problems such as low thyroid, diabetes or nerve disease
- **Pelvic floor problems**: the muscles do not relax to let stool out ("outlet" constipation)
- A colon that moves stool slowly

Outlet constipation is treated mainly with **pelvic floor therapy**, not just laxatives.

Learn the Terms

Constipation

Stools that are infrequent, hard or hard to pass.

Fiber

Plant material that softens stool and adds bulk.

Osmotic laxative

A gentle medicine, such as PEG (polyethylene glycol), that draws water into the stool.

Stimulant laxative

A medicine, such as bisacodyl or senna, that makes the bowel squeeze.

Outlet constipation

Trouble passing stool because the pelvic muscles do not relax.

Biofeedback

Therapy that retrains those muscles to relax during a bowel movement.

Why does my pelvic surgeon care? Straining to pass hard stool pushes down on the same muscles and tissues that hold up the bladder and pelvic organs. So constipation can make prolapse and leakage worse. Treating constipation is often an important step before pelvic surgery, and it can help bladder symptoms too.

How It Is Treated (Step by Step)

- 1 Fiber, fluids and activity:** add fiber slowly (food or a supplement such as psyllium) and drink enough water with it.
- 2 Toilet habits:** go when you feel the urge. Try after a meal. Use a **footstool** so your knees are above your hips, and **do not strain**.
- 3 Gentle laxatives:** an osmotic laxative such as PEG is safe for regular use.
- 4 Pelvic floor therapy or biofeedback** for outlet constipation. In studies, it helped about 7 to 8 in 10 people with this problem.
- 5 Prescription medicines** if store-bought ones do not work.

Good to Know

- Stimulant laxatives are fine for short-term or rescue use. Ask before using them every day for weeks.
- If you take opioid pain medicine, ask your team for a laxative plan.
- If straining is the main problem, more laxatives may not help. Ask about testing and therapy.

Call the office that is treating you if you have:

- Blood in the stool**, or weight loss you cannot explain
- New constipation that does not improve, especially if you are older

Go to an emergency department if:

- You have severe belly pain or vomiting, and cannot pass gas or stool
- You cannot reach the office and are worried

Three Things to Remember

- Straining can make prolapse and bladder leakage worse.
- Start with fiber, fluids, a footstool and a gentle laxative.
- If straining is the main problem, ask about pelvic floor therapy.